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Addressing labour exploitation in the global workforce

For the **Lancet Series on work and health** see <https://www.thelancet.com/series/work-and-health>

We applaud the 2023 *Lancet Series* on work and health for drawing attention to the under-recognised aspects of work that make it a fundamental social determinant of health inequalities. We particularly appreciate the *Series authors'* attention to the low-wage workforce and their expressed concerns about precarity. At the same time, we must also be attuned to the widespread practice of labour exploitation, including extreme exploitation such as human trafficking and hazardous child labour. An estimated 21 million people are in trafficking, bonded, or slave labour situations.¹ Approximately 160 million workers are children, predominantly working in low-income and middle-income countries (LMICs) where child protection mechanisms are often weakest. Similarly, there are an estimated 169 million international migrant workers—and millions more who migrate for work in their own country—who are more likely than the local population to experience exploitative and harmful work conditions, leading to higher rates of injury, physical and mental health problems, and higher mortality rates.²

As the *Series* highlights, impending social, technological, environmental, economic, and political transitions are creating new conditions for exploitation of striving workers. Scholars have described the growing “uberization of work” and workers’ increasing subjection to the “despotism of the algorithm”.³ We believe, however, that with concerted action, these transitions could instead create opportunities for safer, fairer livelihoods and stronger health and social protection measures. A first step in shifting the world’s blindness is to invest in measuring the dimensions of labour exploitation and associated harm, especially in LMICs, where data are especially scarce or underexplored. From a global health perspective, this biased evidence gap is unforgivable given the numbers of unprotected workers who are producing the world’s food supply, building national roads and burgeoning cities, manufacturing our clothing and computer chips, unearthing essential minerals for climate-friendly electric vehicles, and providing essential services.

Although LMIC data on the health protection needs of low-wage workers might be insufficient and imperfect, data are not completely unavailable. For example, occupational injuries could be considered using data sources such as injury surveillance systems, police accident records, compensation registries, and occupational health and safety or labour inspectorate databases. Although we welcome the International Labour Organization (ILO)’s new Occupational Injuries add-on module to their Labour Force Surveys, data collection on specific injury types are missing, and the questions are unlikely to support future estimations of work-related global burden, such as the important WHO–ILO interagency efforts being led by Pega and colleagues.⁴ Moreover, questions on labour exploitation are absent, effectively missing many extreme abuses and broad and severe health outcomes.

Cheap goods have become a luxury for some and a burden for

many. Calculating the global burden of labour exploitation among low-wage workers will not solve the worker health problem but will unquestionably show the potential health gains from instituting inclusive social policies, worker-accessible health services, workplace health and safety regulations, and workers’ rights to collective bargaining against extractive employment terms. As the right to a safe and healthy working environment has been declared a fundamental principle of the rights of workers, now is the time for a serious examination of the costs of harm experienced by our largest and highly undervalued workforce, with greater efforts needed to reduce the global health burden of labour exploitation.

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Inclusive mental health for informal workers

We read with great interest the *Lancet Series* paper on work-related mental health problems by Reiner Rugulies and colleagues.¹ This *Series* paper was a wake-up call for low-income and middle-income countries (LMICs) to

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promote a society in which mental health is a universal human right.

In India, a significant portion of the workforce operates informally, making up more than 90% of the workers. These individuals often work without the protection of regulatory measures. As a result, they do not have job stability, insurance coverage, and legal entitlements, leading to feelings of insecurity and stress. Informal workers in low-paying, precarious employment, especially when working in isolation, face an increased likelihood of encountering psychosocial hazards, which affects their mental wellbeing.² According to the National Crime Records Bureau, 21% of total suicidal deaths were daily wage labourers.³ The Indian Government has implemented measures such as the toll-free helpline Kiran, and the Mental Health and Normalcy Augmentation System application to enhance mental health services.³ However, a substantial gap exists in educating low occupational grade workers about these initiatives. Rural communities have deeply ingrained traditions that stigmatise mental health issues or discourage people from seeking help.⁴

Therefore, including informal workers in labour-related policies and investing in comprehensive research to understand the exact patterns of mental health issues among them is essential. Collecting data on their unique challenges and the effectiveness of implemented policies can inform decision making. Collaboration between governments and non-governmental organisations in both LMICs and high-income countries can mobilise resources, share best practices, and develop holistic strategies for improving mental health in this vulnerable population.

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Challenges for breast radiotherapy with SIB: the IMPORT HIGH trial

Charlotte E Coles and colleagues¹ discuss the latest results from the IMPORT HIGH research, a phase 3 randomised trial that assessed three hypofractionated radiation treatments for efficacy and safety. Most patients were scanned while free breathing, with deep-inspiratory breath-hold techniques implemented only at the end of the trial. This may have biased experimental results. In ischaemic heart disease, pneumonia, and pulmonary fibrosis, considerable delayed unfavourable consequences show the possibility for variance. For example, for ischaemic heart disease, Darby and colleagues found that the number of major coronary events increased linearly with mean heart exposure by 7.4% Gy (95% CI 2.9 to 14.5, $p < 0.001$), with no threshold. The increase started within the first 5 years after radiotherapy and continued into the third decade after radiotherapy.² In addition, our study found that deep-inspiratory breath-hold techniques significantly reduced the mean cardiac dose (standardised mean difference -1.86 , 95% CI -2.26 to 1.46 , $p < 0.01$) and the mean

ipsilateral lung dose in patients with left-sided breast cancer, compared with free breathing.³ Therefore, the two respiratory exercise management modalities might bring about differences in late cardiopulmonary toxicities, especially for patients with left-sided breast cancer.

However, Coles and colleagues did not clarify whether five of the six instances of ischaemic heart disease that occurred were from patients with left-sided breast cancer, and whether this was associated with a difference in respiratory exercise management. It is imperative for the authors to ascertain whether variations in dosing regimens or disparities in respiratory motion management are responsible for the discrepancies in the prevalence of ischaemic heart disease, as this holds substantial clinical significance.

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We read the Article by Charlotte E Coles and colleagues¹ with great interest. The use of hypofractionated simultaneous integrated boost (SIB) radiotherapy with a 48 Gy dose in 15 fractions to the tumour bed showed non-inferiority in ipsilateral local relapse compared with sequential boost. In addition, the lower number of treatments for SIB (15) versus sequential boost (23) have a positive effect on the patients themselves and on the health-care system. To achieve dose localisation,